Content proof for Dr. Gombera, 10/31/2026

Gluteus Tendon Repair: clinical content

Every clinical statement on gluterepair.com, for your sign-off (Gate 1). Each item lists its sources. Highlighted items are my plain-language inferences (9 in all): approve, edit or remove each one.

Status: waiting for your review.

Questions for you

1. Phase IV: functional strengthening starts at month 4, and running, agility and jumping wait until 6 months. Is that split right?
2. Gluteus Maximus Transfer: the pre-op letter does not describe it. Please approve or rewrite the two lines under Your diagnosis.
3. Gluteus Maximus Transfer: after at least 6 weeks, weight bearing as tolerated with a cane during the transition (Phase II, week 7). Correct?
4. The 3D model text: 4 of the 7 part descriptions are mine (highlighted). Please check them.
5. Videos: the fixmyhips hip arthroscopy overview videos are not carried over. Do you have a gluteus repair video to add?

Sources

pt-protocol	PT protocol: Hip Open Gluteus Tendon Repair with/without Bursectomy
discharge	Post-operative instruction checklist: Hip Gluteus Tendon Repair (Open) (discharge sheet)
preop-letter	What to Expect from your Surgery for Hip Gluteus Tendon Repair (pre-op letter)
decisions	Dr. Gombera's decisions, 10/31/2026 (scope section 11)
fixmyhips	fixmyhips.com content he approved on 10/30/2026 (logistics, Phase IV, shared pages)

Where the site follows your decisions, not the printed text

From your decisions of 10/31/2026. Updating the printed documents keeps patients from seeing two answers.

Document	Printed text	Site says
Pre-op letter	PreAdmit 713-794-3525	**(713) 395-6607**
Pre-op letter	4 weeks on crutches or a walker	**Same, plus: some patients need a cane after that**
Pre-op letter	Brace for 4 to 6 weeks when walking	**6 weeks when walking**
Pre-op letter	Pain pills every 3-4 hours as needed	**Norco every 6 hours as needed**
Pre-op letter	6 months (or more) to return to an elite level	**Same, plus: running and high impact at 6 months**
Discharge sheet	20 lb with crutches for 6 weeks	**4 weeks, crutches or a walker**
Discharge sheet	Brace when walking and sitting, for the first 4 weeks	**When walking, for 6 weeks**
Discharge sheet	Sedentary work 4-7 days after surgery	**1 to 2 weeks**
Discharge sheet	Driving after you are evaluated in the office	**Off narcotics, able to lift the leg actively (around 3 weeks), full control**
PT protocol	Brace 4 weeks, 20 lb for 6 weeks	**Brace 6 weeks, 20 lb for 4 weeks**
PT protocol	Phase III weeks 10-15 (overview)	**Weeks 10 to 16**
PT protocol	Running at 3 months	**Running and high impact at 6 months**
PT protocol	Scar massage at portal incisions	**At the incision**

Document	Printed text	Site says
PT protocol	Range-of-motion angles	**Kept for the therapist; patients see only the 90 degree sitting rule**

Notices, word for word

- This is the standard protocol. If you were given a different protocol, follow that one instead.
- Your physical therapist has the final say. These are general suggestions from the standard protocol. If your therapist gives you different exercises, limits or timing, follow your therapist's plan. Stop any exercise that causes sharp pain.
- Your prescriptions may differ. Always take your medications as directed on your own prescription labels.
- Dates are estimates. You move on when your therapist confirms you meet the phase criteria.
- These videos are only a suggestion and are not formal medical advice. The program from your therapist takes precedence, and all exercises should be done under your therapist's observation.
- Continue your home exercise program through 1 year.

1. PT protocol: both procedures side by side

Bold marks where the Gluteus Maximus Transfer differs. The phases and exercises are the same for both.

	Open Gluteus Tendon Repair	Gluteus Maximus Transfer
Weight bearing	20 lb weight bearing, foot flat, with crutches or a walker, for 4 weeks	**20 lb weight bearing, foot flat, with crutches or a walker, for at least 6 weeks**
After that	Wean off the crutches or walker. Some patients need a cane after that.	**Weight bearing as tolerated, usually with a cane during the transition.**
Brace	When walking, 6 weeks	**When walking, 6 to 8 weeks**
Patient limits	No sitting with the hip bent past 90 degrees; No active hip abduction; No passive hip adduction	No sitting with the hip bent past 90 degrees; No active hip abduction; No passive hip adduction
Therapist angles (not	Abduction to 45 degrees, passive only; No extension past 0 degrees;	Abduction to 45 degrees, passive only; No extension past 0 degrees;

	Open Gluteus Tendon Repair	Gluteus Maximus Transfer
shown to patients)	No external rotation past 30 degrees	No external rotation past 30 degrees
Prone lying	2 to 3 hours a day in Phase I	2 to 3 hours a day in Phase I
Aqua therapy	After week 3	After week 3
Running and high impact	6 months	6 months
Driving	Avoid driving until you are no longer taking narcotic pain medication, are able to actively lift your leg (around 3 weeks) and have the limb control to perform all driving tasks.	Avoid driving until you are no longer taking narcotic pain medication, are able to actively lift your leg (around 3 weeks) and have the limb control to perform all driving tasks.
Sources	pt-protocol, decisions	**decisions, pt-protocol**

Phases (Open Gluteus Tendon Repair)

Phase 1: Protection and range of motion (Weeks 1 to 6)

Goals

- Protect the repair
- Avoid active hip abduction and passive hip adduction
- Avoid hip flexor irritation: no sitting with the hip bent past 90 degrees
- Control and decrease pain, inflammation and swelling
- Avoid scar tissue with passive motion and soft tissue mobilization
- Hip brace when walking for 6 weeks
- 20 lb weight bearing, foot flat, for 4 weeks, then wean off crutches or walker; some patients need a cane
- Lie on your stomach 2 to 3 hours a day

Week 1

- Hamstring sets
- Seated hamstring stretch
- Prone lying (lying on your stomach)
- Ankle pumps
- Passive range of motion, lying on your back
- Log rolls
- Isometrics

Week 2

- Circumduction
- Stationary bike, no resistance
- Quadruped cat and camel
- Quadruped rockback
- Quadruped hip extension
- Soft tissue mobilization
- Scar massage at the incision

Note: Continue week 1 exercises

Weeks 3 to 6

- Aquatic therapy
- Double leg bridges
- Stool rotations
- Physioball rollouts

Note: Continue week 2 exercises

Note: After week 4, wean off crutches or walker and normalize your walking pattern

Criteria to progress

- Well-controlled pain after surgery
- No hip or pelvis deviations (limp) when walking

pt-protocol decisions

Phase 2: Stability and gait (Weeks 7 to 9)

Goals

- Protect the repair
- Normalize your walking pattern
- Begin closed chain and weight shift exercises
- Continue Phase I exercises as appropriate

Week 7

- Stationary bike, no resistance

Week 8

- Side-lying clamshells
- Standing hip abduction isometrics

Week 9

- Leg press
- Quadriceps and hamstring strengthening
- Side step-ups
- Y balance reaching
- Balance training
- Elliptical trainer

Note: Leg press: both legs, low weight

Criteria to progress

- No gait deviations
- Minimal pain after activities
- Range of motion equal to the other side

pt-protocol

Phase 3: Strength and muscular endurance (Weeks 10 to 16)

Goals

- Prevent compensation due to fatigue
- Begin resisted biking
- Progress strengthening from double leg to single leg
- Return to prior activities without pain or irritation
- Progress leg strength and endurance

Weeks 10 to 16

- Leg press
- Half prone plank (pillar bridge)
- Double leg body weight squats
- Single leg quarter squat
- Forward, lateral and reverse lunges
- Abductor, IT band and gluteus maximus stretching

Note: Continue all appropriate exercises from Phase II

Note: Leg press: one leg at a time, low weight

Cardio: Elliptical trainer, Resisted biking

pt-protocol decisions

Phase 4: Return to running and activity (Months 4 to 6)

Goals

- No complaints of pain or weakness
- Functional strengthening
- Safe return to your activities
- Maintenance of strength, endurance and balance
- Education about any possible limitations

Weeks 17 to 26

- Balance squats with rotations
- Retro walking with resistance band
- Lunge with trunk rotations

Week 26

- Running progression
- Quick feet
- Backpedaling
- Side shuffles
- Double leg plyometrics
- Z and W cuts
- Cariocas
- Single leg plyometrics
- Return-to-sport and activity exercises

Note: Running, agility and jumping: not before 6 months, and only when your therapist clears you

fixmyhips | pt-protocol | decisions | **Derived: please approve**

Gluteus Maximus Transfer: phase notes that differ

- Phase I goal. Repair: Hip brace when walking for 6 weeks. Transfer: Hip brace when walking for 6 to 8 weeks.
- Phase I goal. Repair: 20 lb weight bearing, foot flat, for 4 weeks, then wean off crutches or walker; some patients need a cane. Transfer: 20 lb weight bearing, foot flat, for at least 6 weeks.
- Phase 1, week 3. Repair: After week 4, wean off crutches or walker and normalize your walking pattern. Transfer: nothing extra.
- Phase 2, week 7. Repair: nothing extra. Transfer: After at least 6 weeks, weight bearing as tolerated, with a cane during the transition.

Exercises (44)

Hamstring sets

Video: Hamstring Sets - Ask Doctor Jo

pt-protocol

Seated hamstring stretch

Caution: Keep within your 90 degree hip limit

Video: NUHS Physiotherapy - Hamstring stretch in sitting

pt-protocol

Prone lying (lying on your stomach)

- Prone hangs
- Lie on your stomach 2 to 3 hours a day

Video: Prone Lying Hip Extension | Hip Replacement Post-Op Physio & Exercises

pt-protocol

Ankle pumps

- Perform continuously throughout the day

Video: Ankle Pumps

pt-protocol discharge

Passive range of motion, lying on your back (done with the therapist)

For the therapist only: Flexion to 75 degrees; Abduction to 30 degrees; External and internal rotation to 20 degrees, with the hip flexed; 30 repetitions in each plane of motion

Video: Passive Range of Motion (PROM) Flexion

pt-protocol

Log rolls

Video: none yet (shows Video coming soon)

pt-protocol

Isometrics

- Focus on the deep abdominals, obliques, gluteals and abductors

Video: Transverse Abdominus Isometric; Glute Sets | Physical Therapy Exerscises; How to Do a Hooklying Isometric Hip Abduction with Belt Exercise | MedBridge

pt-protocol

Circumduction (done with the therapist)

- Gently move the thigh in small circles, first clockwise, then counterclockwise
- Avoid rotation
- 5 minutes in each direction

For the therapist only: Hip flexed to 70 degrees, knee flexed to 90 degrees; Repeat in neutral: hip abducted 20 degrees, knee extended

Video: Caregiver-assisted Circumduction Passive Range of Motion (PROM); Circumduction in Neutral -Passive Range of Motion (PROM)

pt-protocol

Stationary bike, no resistance

- No resistance
- Keep the seat high
- No recumbent bike

Video: Stationary Bike after Hip Arthroscopic Surgery

pt-protocol

Quadruped cat and camel

- Be mindful of your range of motion limits

Video: Cat Camel

pt-protocol

Quadruped rockback

- With a slight backward pelvic tilt
- Be mindful of your range of motion limits

Video: Quadruped rocking

pt-protocol

Quadruped hip extension

- Be mindful of your range of motion limits

Video: Quadruped Hip Extension

pt-protocol

Soft tissue mobilization (done with the therapist)

- Start superficial, progress to deep

Video: none yet (shows Video coming soon)

pt-protocol

Scar massage at the incision

- 5 minutes

Caution: Only once the incision is healed

Video: Get Rid of Scar Tissue - Ask Doctor Jo

pt-protocol decisions

Aquatic therapy

- May begin after week 3, as long as the incision is well healed

Video: none yet (shows Video coming soon)

pt-protocol

Double leg bridges

Video: How to: Bridge exercise by the Women's Sports Medicine Program at Mass General

pt-protocol

Stool rotations

- Within your range of motion limits

Video: none yet (shows Video coming soon)

pt-protocol

Physioball rollouts

Video: Tall Kneeling Stability Ball Roll Out

pt-protocol

Side-lying clamshells

- Progress to TheraBand resistance

Video: How to Do a Clamshell Exercise | MedBridge

pt-protocol

Standing hip abduction isometrics

- Against a foam roller or wall

Video: How to Do a Standing Isometric Hip Abduction with Knee at Wall Exercise | 30 Seconds | MedBridge

pt-protocol

Leg press

- Low weight
- Both legs first (week 9), then one leg at a time (Phase III)

Video: Partial Weight Bearing Single Leg Leg Press

pt-protocol

Quadriceps and hamstring strengthening (done with the therapist)

- Advance as directed by your therapist

Video: none yet (shows Video coming soon)

pt-protocol

Side step-ups

Video: How to Do a Lateral Step-up Exercise | 30 Seconds | MedBridge

pt-protocol

Y balance reaching

Video: Y Balance Exercise

pt-protocol

Balance training

- Bosu ball, balance board, single leg stance

Video: Single Leg Balance Progression

pt-protocol

Half prone plank (pillar bridge)

- Progress to full plank or Bosu when appropriate

Video: How to Do a Plank: A Guide from Physical Therapists

pt-protocol

Double leg body weight squats

Video: Squats - Ask Doctor Jo

pt-protocol

Single leg quarter squat

Video: How To Do a Single Leg Quarter Squat | 30 Seconds | MedBridge

pt-protocol

Forward, lateral and reverse lunges

Video: How to Do Lunges: A Guide from Physical Therapists; How to Do Side Lunges: A Guide from Physical Therapists

pt-protocol

Abductor, IT band and gluteus maximus stretching

Video: How to Do a Glute Stretch: A Guide from Physical Therapists; IT Band Stretch, Standing - Ask Doctor Jo

pt-protocol

Elliptical trainer

- Start with 5 minutes, increase 5 minutes each week

Video: Ask the Expert: Proper Use of an Elliptical

pt-protocol

Resisted biking

- Advance resistance as tolerated

Video: none yet (shows Video coming soon)

pt-protocol

Balance squats with rotations

Video: Rotational Squat #balanceexercise #legstrengthening #multiplesclerosis

fixmyhips pt-protocol

Retro walking with resistance band

Video: Bulletproof Fitness: Resisted Retro Walk

fixmyhips pt-protocol

Lunge with trunk rotations

- With a sport cord or physioball

Video: Alternating Lunge with a Trunk Twist Exercise for Rotational Strength & Stability

fixmyhips pt-protocol

Running progression

- Not before 6 months
- Begin with single plane agilities

Video: Return to Running After Hip Surgery

fixmyhips pt-protocol decisions

Quick feet

- Not before 6 months

Video: ACL Recovery | Ladder Drills

fixmyhips pt-protocol

Backpedaling

- Not before 6 months

Video: Agility Exercise for ACL: Forward Backward Jog Stops

fixmyhips pt-protocol

Side shuffles

- Not before 6 months

Video: Agility Exercise for ACL: Lateral Shuffle

fixmyhips pt-protocol

Double leg plyometrics

- Not before 6 months
- For example broad jumps and 4-square hops

Video: Hopping & Jumping Progressions (Rehab & Training) #shorts; 4 Square Hops

fixmyhips pt-protocol decisions

Z and W cuts

- Not before 6 months

Video: Early Cutting Drills without an ACL

fixmyhips pt-protocol decisions

Cariocas

- Not before 6 months

Video: Running Drill - Carioca - Ask Dr. Abelson

fixmyhips pt-protocol

Single leg plyometrics

- Not before 6 months

Video: Single-Leg Hop Stability Progression Using A Yoga Mat

fixmyhips | pt-protocol | decisions

Return-to-sport and activity exercises (done with the therapist)

Video: none yet (shows Video coming soon)

fixmyhips | pt-protocol

2. Post-operative instruction checklist: Hip Gluteus Tendon Repair (Open) (discharge sheet)

Before surgery checklist

Get an ice machine or ice packs

- Icing starts right after surgery: 30 to 45 minutes every 2 hours until your first post-op visit. An ice machine is optional and may or may not be covered by your insurance, so check with your insurance company. Ice packs work if you do not get a machine.

discharge | decisions | fixmyhips

Surgery day

After surgery

- Ice starts right away.
- You will walk with crutches or a walker, putting only 20 lb on the operated leg, and wear the brace when walking.
- Most patients spend one night in the hospital.

discharge | decisions | preop-letter

Home care

Dressing

- Keep your surgical dressing on. You may remove the outer bulky dressing 48 hours after surgery.
- It is normal for the hip to bleed, swell or bruise after surgery.
- Keep the waterproof bandage on until your first post-op visit.
- If dressing supplies were sent to you, remove the waterproof dressing and apply the new dressings starting on day 4 after surgery.
- Do not put ointments or creams on the incision.

discharge

Home care

Showering

- Keep the incision clean and dry. The waterproof dressing can stay on when you shower.
- You may shower 48 hours after surgery.
- Do not soak the operated leg, for example in a bath.
- Take the brace off to shower.

discharge

Home care

Ice

- Start icing right after surgery.
- Use the ice machine, or ice packs if you did not get a machine, for 30 to 45 minutes every 2 hours until your first post-op visit.
- Protect your skin to avoid frostbite.
- You do not need to wake up at night to change the ice unless you are uncomfortable.

discharge decisions

Home care

Activity

- Raise your leg to chest level whenever possible to decrease swelling.
- Walk with crutches or a walker, putting only 20 lb on the operated leg, foot flat, for 4 weeks (at least 6 weeks after a Gluteus Maximus Transfer). Then wean off them as your therapist directs. Some patients need a cane after that.
- It is safe to bend the hip within the brace limits. Do not bend the hip past 90 degrees.
- Do ankle pumps continuously throughout the day.
- Avoid activities that increase hip pain, such as long periods of standing or walking.
- Avoid long periods of sitting or long-distance travel for 2 weeks after surgery.
- Formal physical therapy usually begins 3 to 5 days after surgery. Your prescription and protocol come with your discharge paperwork.

discharge | decisions

Home care

First post-op visit

- Usually 10 to 14 days after surgery, with our Physician Assistant.
- She will check your incision, go over the surgery and your recovery, and answer your questions.
- The incision is closed with buried sutures, so no stitches need to be removed.
- To reschedule, call (713) 794-3457.

discharge | preop-letter

Medications

Standard regimen

- Baby aspirin (81 mg), 1 tablet twice a day for 2 weeks, to lower the risk of a blood clot.
- Norco, 1 tablet every 6 hours as needed for pain. Do not take more than 4,000 mg of acetaminophen in 24 hours.
- Naprosyn, 1 tablet every 12 hours with food (anti-inflammatory). Tell us if you cannot take it or other NSAIDs. It is OK with aspirin. Do not combine it with other NSAIDs such as ibuprofen, Advil, Aleve or etodolac.
- Robaxin, 1 tablet every 12 hours as needed, for the first week. Helps with pain and muscle spasm.

discharge | decisions

Medications

Taking pain medication

- Your pain medication prescription is given to you on the day of surgery. Texas law requires a written copy, so strong pain medication cannot be called in.
- Take pain medication with food to reduce nausea. You do not have to take it if rest, repositioning or ice controls the pain.
- Common side effects are nausea, drowsiness and constipation. For constipation, an over-the-counter laxative and plenty of water can help.
- If you have problems with nausea and vomiting, contact the office.
- Do not drive or operate machinery, and avoid alcohol, while taking narcotic pain medication.
- For refills, contact your pharmacy and they will call the office. We cannot refill after 5 p.m. or refill narcotic medication on weekends.

discharge | preop-letter

When to call

Call us if you have

- Painful swelling or numbness
- Pain that will not let up
- Fever over 101°F, or chills
- Continuous drainage or bleeding from the incision
- Difficulty breathing
- Excessive nausea or vomiting
- Calf pain

discharge

When to call

Usually normal

- Some swelling and numbness
- Bleeding, swelling or bruising around the hip
- Swelling in the groin, buttock and thigh, and bruising on the outer leg
- A small amount of drainage from the incision
- A low-grade temperature
- More pain on day 1 or 2 after surgery
- Stiffness and discomfort for a few days

discharge | preop-letter

FAQ, after surgery

When can I shower?

- 48 hours after surgery. The waterproof dressing can stay on in the shower. Take the brace off to shower. No baths or soaking.

discharge

FAQ, after surgery

What if I was sent dressing supplies?

- Remove the waterproof dressing and apply the new dressings starting on day 4 after surgery. Otherwise, keep the waterproof bandage on until your first post-op visit.

discharge

FAQ, after surgery

How far can I bend my hip?

- Do not bend the hip past 90 degrees, for example in a low chair. Avoid long periods of sitting for 2 weeks after surgery.

discharge decisions

FAQ, after surgery

Will I need physical therapy?

- Yes. Formal physical therapy usually begins 3 to 5 days after surgery. The protocol runs in four phases over about 6 months, and your home exercise program continues through 1 year.

discharge pt-protocol

FAQ, after surgery

When is my first visit after surgery?

- Usually 10 to 14 days after surgery, with our Physician Assistant. No stitches need removing because the sutures are buried under the skin.

discharge preop-letter

3. What to Expect from your Surgery for Hip Gluteus Tendon Repair (pre-op letter)

Before surgery checklist

Pre-register online

- Pre-registration must be completed no less than 7 days before surgery. At texasorthopedic.com, open the Menu, go to Patients and Visitors, choose Online Pre-Registration, then under MyHealthOne choose Create your account or Sign in. You can also complete it at your PreAdmit visit.

preop-letter

Before surgery checklist

Complete PreAdmit testing (mandatory)

- PreAdmit is required for clearance to receive anesthesia. It is on the 3rd floor of Texas Orthopedic Hospital, where the nursing and anesthesiology teams will interview you and order any tests needed, such as blood tests or an EKG. Complete it within 30 days of surgery and at least one week before. Call (713) 395-6607 to schedule, or walk in 8 a.m. to 5 p.m., Monday through Friday.

preop-letter decisions

Before surgery checklist

Arrange a ride home and help at home

- You will typically spend one night in the hospital, and you cannot drive while taking narcotic pain medication, so arrange a ride home.

preop-letter discharge **Derived: please approve**

Before surgery checklist

Plan time off work

- For most office jobs, plan 1 to 2 weeks off. Recovery is generally complete, and heavy lifting possible, at 4 to 6 months.

preop-letter decisions

Before surgery checklist

Pack for one night

- You will typically spend one night in the hospital, so pack for one night.

preop-letter **Derived: please approve**

Surgery day

Arrive in the morning

- You come to Texas Orthopedic Hospital in the morning, have surgery, and typically spend one night in the hospital.
- Your arrival time comes by phone the day before. No call by 3 p.m.? Call (713) 794-3457.

preop-letter fixmyhips

Surgery day

Sign the consent

- A nurse will have you sign the operative permit. It will read "Hip Gluteus Repair." Additional procedures may be included depending on what is found in or around your hip.

preop-letter

Surgery day

Anesthesia

- The surgery is almost always performed under general anesthesia, sometimes with an additional regional anesthetic block.
- You will meet the anesthesia team at PreAdmit and again on the day of surgery.

preop-letter

Surgery day

Surgery

- A small incision is made on the outer part of your hip.
- The irritated bursa is removed if present, and a small window (fasciotomy) is made in the IT band so the gluteus tendon can be seen directly.
- Small anchors are used to sew the tendon back down to the bone. This secures the repair and lets the tendon heal.
- Surgery usually lasts 40 to 80 minutes, sometimes more.
- At the end, medication may be injected to reduce pain. The incision is closed with buried sutures, so no stitches need to be removed.

preop-letter

Home care

Take care with these for up to 8 weeks

- Prolonged standing, especially on hard surfaces
- Prolonged walking, such as around shopping centers
- Heavy lifting
- Squatting or crouching

preop-letter

Home care

Returning to work

- For most office jobs, plan 1 to 2 weeks off. Your hip will be sore: do no lifting, pushing, pulling or carrying.
- Recovery is generally complete, and heavy lifting possible, at 4 to 6 months.
- There are no fixed rules for return to work; these are guidelines for you and your employer.

preop-letter decisions

Home care

What to expect

- Discomfort and stiffness are normal for a few days. Pain may be felt in the hip, lower back, buttock, knee and ankle.
- Pain is often worse on day 1 or 2, when swelling peaks.
- Swelling in the groin, buttock and thigh, and bruising on the outer leg, are common. They improve over the following days.
- Most people walk relatively pain-free 8 to 10 weeks after surgery.
- Running and high impact activity wait until 6 months. Returning to an elite level of fitness can take 6 months or more.
- Any unexpected increase in pain can be treated with ice packs and anti-inflammatory medication.

preop-letter discharge decisions

Your diagnosis

Results and risks

- This surgery is successful about 85 to 90 percent of the time. It is most successful at relieving pain.
- Returning to vigorous work or sport depends on the damage to your hip, how well it heals, how well you rehabilitate and how demanding your activity is.
- Complications are rare (less than 1 percent), and most are temporary. For example, the infection rate after hip surgery is less than 1 in 1,000.
- Possible complications include re-tear of the repair, infection, fracture, increased pain, impotence, bleeding, nerve injury, abandoned procedure, blood clot in the leg, instrument breakage, avascular necrosis of the femoral head, delayed wound healing, worsening symptoms, arthritis, stiffness, need for further surgery, and the risks of anesthesia itself.

preop-letter

Your diagnosis

The gluteus medius and minimus

- Muscles on the outside of the hip that move the leg out to the side (abduction).
- They help you stand upright and walk without a limp.

preop-letter

Your diagnosis

Gluteus medius and minimus tears

- Sometimes called the rotator cuff tear of the hip.
- The tendon tears away from where it attaches to the greater trochanter, often called the "lateral hip bone."
- A tear can cause lasting pain that feels like trochanteric bursitis. It can also cause weakness and limping.
- A tear should be suspected when physical therapy and injections into the trochanteric bursa do not give lasting relief.

preop-letter

Your diagnosis

Open Gluteus Tendon Repair

- In many cases the torn tendon can be sewn back to the bone using tiny suture anchors.
- This surgery has a high success rate in treating pain, and may restore strength to the gluteus muscle.

preop-letter

Your diagnosis

Why surgery, and the alternatives

- Surgery is recommended for persistent pain that interferes with daily life, work or sport.
- Alternatives range from no treatment to physical therapy, medication or injections. These can be arranged if you prefer.

preop-letter

3D anatomy model

Greater trochanter

- The bump of bone on the outside of the hip, often called the "lateral hip bone." The gluteus tendons attach here.

preop-letter

3D anatomy model

Gluteus medius

- A muscle on the outside of the hip that moves the leg out to the side and helps you walk without a limp. Its tendon can tear away from the bone.

preop-letter

3D anatomy model

Gluteus minimus

- A smaller muscle under the gluteus medius. It works with the gluteus medius and can tear with it.

preop-letter **Derived: please approve**

3D anatomy model

Footprint

- The area of bone where the tendons attach. In a tear, the tendon pulls away and leaves the bone bare. The repair holds the tendon back down on it so it can heal.

preop-letter **Derived: please approve**

3D anatomy model

Trochanteric bursa

- A small cushion over the greater trochanter. It can become irritated. If it is irritated, it is removed during surgery.

preop-letter Derived: please approve

3D anatomy model

IT band

- A thick band of tissue on the outside of the thigh. A small window (fasciotomy) is made in it during surgery to reach the tendon.

preop-letter Derived: please approve

3D anatomy model

Anchors and sutures

- Small anchors in the bone hold sutures that sew the tendon back down.
- Two anchors sit higher up (the inner row) and one or two lower down (the outer row). The sutures cross between them and press the tendon flat onto the bone.

preop-letter decisions

FAQ, before surgery

How long does the surgery take?

- Usually 40 to 80 minutes, sometimes more.

preop-letter

FAQ, before surgery

Will I stay in the hospital?

- Yes. You come in the morning, have surgery, and typically spend one night in the hospital.

preop-letter

FAQ, before surgery

Will I be asleep?

- Yes. The surgery is almost always done under general anesthesia, sometimes with an additional regional anesthetic block. You will meet the anesthesia team at PreAdmit and again on the day of surgery.

preop-letter

FAQ, after surgery

When can I go back to work?

- For most office jobs, after 1 to 2 weeks, with no lifting, pushing, pulling or carrying. Heavy lifting usually takes 4 to 6 months.

preop-letter decisions

FAQ, after surgery

When will walking stop hurting?

- In most cases, you should be walking relatively pain-free 8 to 10 weeks after surgery.

preop-letter

4. Dr. Gombera's decisions, 10/31/2026 (scope section 11)

Before surgery checklist

Set up your home for crutches or a walker

- You will walk with crutches or a walker, putting only 20 lb on the operated leg, for at least 4 weeks. Have a place ready to keep your leg raised to chest level.

decisions discharge **Derived: please approve**

Home care

Brace

- Wear the brace when walking for 6 weeks (6 to 8 weeks after a Gluteus Maximus Transfer).
- It can come off to shower, during exercises and at night.

decisions discharge

Home care

Driving

- Do not drive while taking narcotic pain medication.
- You can start driving once you are off narcotic pain medication, can actively lift your leg (around 3 weeks), and have full control of the leg for all driving tasks.

decisions | pt-protocol | discharge

Your diagnosis

Gluteus Maximus Transfer

- Used when the gluteus medius and minimus tendons cannot be repaired back to the bone. Part of the gluteus maximus is moved over to do their job.
- Recovery follows the same protocol, with longer protection: 20 lb weight bearing for at least 6 weeks and the brace for 6 to 8 weeks.

decisions | **Derived: please approve**

FAQ, after surgery

How long will I need crutches or a walker?

- 4 weeks, putting only 20 lb on the operated leg with your foot flat. After a Gluteus Maximus Transfer, at least 6 weeks. Then you wean off them; some patients need a cane after that.

decisions

FAQ, after surgery

How long will I wear the brace?

- When walking, for 6 weeks (6 to 8 weeks after a Gluteus Maximus Transfer). It can come off to shower, during exercises and at night.

decisions | discharge

FAQ, after surgery

When can I drive?

- Once you are off narcotic pain medication, can actively lift your leg (around 3 weeks), and have full control of the leg for all driving tasks.

decisions | pt-protocol

FAQ, after surgery

When can I run or play sports?

- Running and high impact activity wait until 6 months. Returning to an elite level of fitness can take 6 months or more, and your therapist guides your return.

decisions | preop-letter

5. fixmyhips.com content he approved on 10/30/2026 (logistics, Phase IV, shared pages)

Before surgery checklist

Tell the anesthesia team your history

- If you are prone to nausea or vomiting after anesthesia, or have had complications with anesthesia, let the anesthesia team know.

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Before surgery checklist

Optional: decide on a biologic add-on

- PRP, stem cell or connective tissue matrix treatment can be added during surgery. It is not covered by insurance. Call the office for pricing before surgery day.

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Before surgery checklist

Nothing to eat or drink after midnight

- PreAdmit will tell you which medications to stop and when. Make sure they know if you take a GLP-1 medication (such as Ozempic, Wegovy, Mounjaro or Zepbound) or any blood thinner.

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Before surgery checklist

Expect a call with your arrival time

- The office calls the day before surgery with your arrival time. If you have not heard from us by 3 p.m. that day, call (713) 794-3457.

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Add-ons

Biologic add-ons: PRP, stem cell and connective tissue matrix

- Stem cells are used to repair diseased or damaged tissues.
- Stem cell, connective tissue matrix or PRP injections may not heal the tendon on their own, but there is emerging evidence they can augment and strengthen your surgical repair.
- These treatments can be done during your surgery.
- They are currently not covered by insurance.
- Call for pricing: (713) 794-3457, or email info@gomberamd.com.

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FAQ, before surgery

Can I eat or drink before surgery?

- Nothing to eat or drink after midnight. PreAdmit will tell you which medications to stop, especially GLP-1 medications and blood thinners.

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FAQ, before surgery

When do I get crutches and a brace?

- Usually on the day of surgery. If insurance does not cover the brace, we will discuss alternatives with you.

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